NUR 774 Exam 3: 100-Question Practice Exam

70 single-answer + 30 Select All That Apply (SATA). Built from your Integrated Study Guide. SATA tip: judge each option as its own true/false question.

How to use: read the question, pick your answer, then swipe to the next page for the answer and rationale.

Question 1 of 100

SELECT ALL THAT APPLY

Which vaccines are routinely given at the 2-month visit?

A. PCV

B. MMR

C. Rotavirus

D. DTaP

E. Hib

F. IPV

G. Varicella

Swipe for answer →

Answer 1

A, C, D, E, F

- **PCV**
- **Rotavirus**
- **DTaP**
- **Hib**
- **IPV**

Why: "Dad Is Having Pizza Rolls." MMR and varicella start at 12–15 months.

Immunizations

SELECT ALL THAT APPLY

When is amoxicillin-clavulanate preferred over amoxicillin for AOM?

A. Recurrent AOM that failed amoxicillin

B. First ear infection with no recent antibiotics

C. Purulent conjunctivitis with the ear infection

D. Amoxicillin taken in the past 30 days

E. History of penicillin anaphylaxis

Swipe for answer →

Answer 2

A, C, D

- **Recurrent AOM that failed amoxicillin**
- **Purulent conjunctivitis with the ear infection**
- **Amoxicillin taken in the past 30 days**

Why: Augmentin 3 C's: 30 days, Conjunctivitis, Can't clear. Penicillin anaphylaxis needs a non-penicillin choice.

Otitis media

Question 3 of 100

SELECT ALL THAT APPLY

During an asthma exacerbation, which findings suggest impending respiratory failure?

A. Speaking in full sentences

B. SpO2 of 98%

C. Less wheezing with poor air movement

D. Cyanosis

E. Increasing fatigue

F. Altered responsiveness

Swipe for answer →

Answer 3

C, D, E, F

- **Less wheezing with poor air movement**
- **Cyanosis**
- **Increasing fatigue**
- **Altered responsiveness**

Why: "A silent chest is a scream." A quieter chest can mean less air is moving.

Asthma

Question 4 of 100

SINGLE ANSWER

A parent asks why their child must finish all 10 days of amoxicillin for strep throat when they already feel better. The best response is that completing the course:

A. Prevents infectious mononucleosis

B. Reduces the risk of acute rheumatic fever

C. Prevents poststreptococcal glomerulonephritis

D. Is needed only if the child has scarlet fever

Swipe for answer →

Answer 4

B

- **Reduces the risk of acute rheumatic fever**

Why: "Penicillin protects the Heart, not the Kidney." Treatment prevents ARF; antibiotics do not reliably prevent PSGN.

Strep pharyngitis

Question 5 of 100

SINGLE ANSWER

Which is an example of CONDUCTIVE hearing loss?

A. Congenital CMV damage to hair cells

B. Damage to the cochlea

C. Hearing loss from middle-ear fluid (OME)

D. Auditory nerve damage

Swipe for answer →

Answer 5

C

- **Hearing loss from middle-ear fluid (OME)**

Why: Conductive = Cerumen and fluid (outer/middle ear). Sensorineural = Snail (cochlea) and nerve.

Ear and hearing

SELECT ALL THAT APPLY

Which interventions are appropriate for a child with croup?

A. Albuterol nebulizer

B. Single dose of dexamethasone

C. Routine neck x-ray

D. Keeping the child calm with the caregiver

E. Amoxicillin

F. Nebulized epinephrine for stridor at rest

Swipe for answer →

Answer 6

B, D, F

- **Single dose of dexamethasone**
- **Keeping the child calm with the caregiver**
- **Nebulized epinephrine for stridor at rest**

Why: "Dex for all, Epi for stridor at rest." Albuterol and antibiotics don't treat croup. Croup is a clinical diagnosis.

Croup

Question 7 of 100

SINGLE ANSWER

A 9-year-old had a sore throat 3 weeks ago and now has migratory knee and ankle arthritis plus a new murmur. ASO is elevated. The most likely diagnosis is:

A. Acute rheumatic fever

B. Kawasaki disease

C. PSGN

D. Septic arthritis

Swipe for answer →

Answer 7

A

- **Acute rheumatic fever**

Why: JONES: 2 majors (Joints + carditis) plus evidence of prior strep = ARF.

Rheumatic fever

Question 8 of 100

SINGLE ANSWER

A 44-lb child with AOM is ordered amoxicillin 90 mg/kg/day divided BID. How many mg per dose?

A. 1,980 mg

B. 900 mg

C. 1,800 mg

D. 450 mg

Swipe for answer →

Answer 8

B

- **900 mg**

Why: 44 ÷ 2.2 = 20 kg. 20 × 90 = 1,800 mg/day ÷ 2 = 900 mg per dose. "Per DAY → Divide."

Dose calculation

SINGLE ANSWER

A child has intensely itchy eyes in both eyes, with watery, stringy discharge, during spring. What should the nurse teach?

A. This is not contagious; use cool compresses and allergy eye drops

B. Wear an eye patch

C. Stay home until 24 hours of antibiotic drops

D. Start antibiotic ointment

Swipe for answer →

Answer 9

A

- **This is not contagious; use cool compresses and allergy eye drops**

Why: Allergic = Itchy. Not contagious. Reduce exposure; antihistamine/mast-cell stabilizer drops.

Eye

Question 10 of 100

SINGLE ANSWER

A fully immunized 4-year-old has abrupt high fever, tachypnea, and focal crackles in the right lower lobe. The child is stable for outpatient care. The expected treatment is:

A. No antibiotics, supportive care only

B. High-dose amoxicillin, about 90 mg/kg/day divided BID

C. Daily IM ceftriaxone for 10 days

D. Azithromycin alone

Swipe for answer →

Answer 10

B

- **High-dose amoxicillin, about 90 mg/kg/day divided BID**

Why: Typical bacterial pneumonia (S. pneumoniae) → amoxicillin. A macrolide is not an automatic substitute.

Pneumonia

SINGLE ANSWER

A 16-week-old infant has never received rotavirus vaccine. What should the nurse do?

A. Wait until 6 months to start

B. Do not start rotavirus; the first dose must be given by 14 weeks 6 days

C. Give a double dose to catch up

D. Give rotavirus dose 1 today

Swipe for answer →

Answer 11

B

- **Do not start rotavirus; the first dose must be given by 14 weeks 6 days**

Why: Rotavirus: start by 14 weeks 6 days ("by 15 weeks"), finish by 8 months.

Immunizations

Question 12 of 100

SELECT ALL THAT APPLY

A child has pneumonia. Which findings require urgent/hospital evaluation?

A. Hypoxemia

B. Apnea

C. Temperature of 38.5°C with normal work of breathing

D. Cough for 3 days

E. Central cyanosis

F. Altered responsiveness

G. Inability to take oral medication

Swipe for answer →

Answer 12

A, B, E, F, G

- **Hypoxemia**
- **Apnea**
- **Central cyanosis**
- **Altered responsiveness**
- **Inability to take oral medication**

Why: Urgent findings: apnea, central cyanosis, hypoxemia, severe work of breathing, altered responsiveness, toxic look, dehydration, or can't take meds.

Pneumonia

Question 13 of 100

SINGLE ANSWER

The long-term cardiac damage from rheumatic fever most often affects the:

A. Conduction system only

B. Coronary arteries

C. Pericardium

D. Mitral (and aortic) valves

Swipe for answer →

Answer 13

D

- **Mitral (and aortic) valves**

Why: Rheumatic = Valves. Kawasaki = Coronaries.

Rheumatic fever

Question 14 of 100

SINGLE ANSWER

An 11-year-old has 10 days of gradually worsening headache, malaise, and dry cough but looks well. Which medication does the nurse anticipate?

A. Azithromycin

B. Amoxicillin

C. Oseltamivir

D. Cephalexin

Swipe for answer →

Answer 14

A

- **Azithromycin**

Why: "My School = Mycoplasma." A gradual, atypical pattern in a school-age child is treated with a macrolide.

Pneumonia

SINGLE ANSWER

After 72 hours of amoxicillin for pneumonia, a child is still febrile and slightly worse. The best nursing action is to:

A. Switch to a nebulized bronchodilator

B. Continue the same plan for 7 more days

C. Report for reassessment of complications, adherence, dose, or another diagnosis

D. Add an over-the-counter cough suppressant

Swipe for answer →

Answer 15

C

- **Report for reassessment of complications, adherence, dose, or another diagnosis**

Why: No improvement in 48–72 hours → reassess for empyema/effusion, the wrong organism, adherence, or a foreign body.

Pneumonia

SINGLE ANSWER

A 6-week-old is afebrile with a staccato cough and tachypnea. The infant had conjunctivitis at 2 weeks of age. The most likely organism is:

A. Group B Streptococcus

B. Staphylococcus aureus

C. Mycoplasma pneumoniae

D. Chlamydia trachomatis

Swipe for answer →

Answer 16

D

- **Chlamydia trachomatis**

Why: Chlamydia's 3 C's: Choppy (staccato) cough, Cool (afebrile), Conjunctivitis first. Classic at 1–3 months.

Pneumonia

SELECT ALL THAT APPLY

Which findings suggest foreign body aspiration?

A. Wheeze in both lungs after a 3-day cold

B. Sudden choking or coughing while eating

C. Decreased breath sounds on one side

D. Wheeze on one side only

E. Repeated pneumonia in the same lung area

F. A normal x-ray, which rules it out

Swipe for answer →

Answer 17

B, C, D, E

- **Sudden choking or coughing while eating**
- **Decreased breath sounds on one side**
- **Wheeze on one side only**
- **Repeated pneumonia in the same lung area**

Why: "Sudden + one Side." Many foods don't show on x-ray, so a normal film does not rule it out.

Foreign body

SINGLE ANSWER

A child has migratory polyarthritis, fever, and elevated ESR. Why is arthralgia NOT counted as a minor criterion here?

A. Arthralgia is a major criterion

B. Arthralgia only counts in adults

C. Arthralgia cannot be counted when arthritis is already a major criterion

D. Arthralgia requires an ASO titer

Swipe for answer →

Answer 18

C

- **Arthralgia cannot be counted when arthritis is already a major criterion**

Why: Don't double-count: arthritis + arthralgia, or carditis + prolonged PR.

Rheumatic fever

Question 19 of 100

SINGLE ANSWER

A harsh holosystolic murmur at the lower left sternal border is most consistent with:

A. Atrial septal defect

B. Still's murmur

C. Patent ductus arteriosus

D. Ventricular septal defect

Swipe for answer →

Answer 19

D

- **Ventricular septal defect**

Why: VSD = "Very loud." And a small VSD can be loud: "Small hole, loud howl."

ASD/VSD

SELECT ALL THAT APPLY

Which findings are consistent with a peritonsillar abscess?

A. Marked one-sided throat pain

B. Trismus

C. Barking cough

D. Uvula deviated away from the affected side

E. Sandpaper-like rash

F. Muffled "hot potato" voice

Swipe for answer →

Answer 20

A, B, D, F

- **Marked one-sided throat pain**
- **Trismus**
- **Uvula deviated away from the affected side**
- **Muffled "hot potato" voice**

Why: "The uvula runs AWAY from the abscess." It needs urgent assessment and often drainage plus antibiotics.

Strep complications

Question 21 of 100

SINGLE ANSWER

Which presentation meets the criteria for acute bacterial sinusitis?

A. Sneezing, itchy eyes, and clear drainage

B. Clear rhinorrhea for 5 days with low-grade fever

C. Daytime cough and nasal drainage for more than 10 days without improvement

D. Green nasal mucus for 3 days

Swipe for answer →

Answer 21

C

- **Daytime cough and nasal drainage for more than 10 days without improvement**

Why: "10 days, Double-dip, 39 for 3." Colored mucus alone is not enough.

Other respiratory

Question 22 of 100

SINGLE ANSWER

A child has sharp chest pain that worsens lying flat and improves leaning forward. A friction rub is heard. The nurse suspects:

A. Costochondritis

B. Pericarditis

C. Aortic stenosis

D. Myocarditis

Swipe for answer →

Answer 22

B

- **Pericarditis**

Why: Pericarditis = "praying position" relief. Assess for effusion/tamponade.

Other cardiac

SELECT ALL THAT APPLY

In which children with AOM are antibiotics generally indicated?

A. A child with new ear drainage (otorrhea)

B. A child with a temperature of 39.2°C

C. An 18-month-old with AOM in both ears

D. A 4-month-old with confirmed AOM

E. A 3-year-old with mild one-sided pain for 1 day and a temperature of 38.1°C

F. A child with ear pain for 3 days

Swipe for answer →

Answer 23

A, B, C, D, F

- **A child with new ear drainage (otorrhea)**
- **A child with a temperature of 39.2°C**
- **An 18-month-old with AOM in both ears**
- **A 4-month-old with confirmed AOM**
- **A child with ear pain for 3 days**

Why: Under 6 months, under 2 years with both ears, or severe (otorrhea, 39°C or more, pain 48 hours or more) → antibiotics. Age 2+ nonsevere = watch or treat.

Otitis media

SINGLE ANSWER

A 3-year-old received DTaP doses 1 and 2 as an infant, then missed a year. What is the correct plan?

A. No more doses are needed

B. Continue with dose 3; do not restart the series

C. Restart the series from dose 1

D. Give Tdap instead

Swipe for answer →

Answer 24

B

- **Continue with dose 3; do not restart the series**

Why: Catch-up: NEVER restart a series. DTaP is used under 7 years.

Immunizations

Question 25 of 100

SINGLE ANSWER

An infant with a large VSD had a normal newborn exam but develops tachypnea and poor feeding at 6 weeks. Why?

A. Pulmonary vascular resistance fell, increasing left-to-right shunting

B. The ductus arteriosus reopened

C. The shunt reversed to right-to-left

D. The VSD grew larger

Swipe for answer →

Answer 25

A

- **Pulmonary vascular resistance fell, increasing left-to-right shunting**

Why: As pulmonary resistance drops over the first weeks, more blood crosses to the lungs → heart failure.

ASD/VSD

Question 26 of 100

SELECT ALL THAT APPLY

A child has a red eye. Which findings need urgent, broader evaluation?

A. True photophobia

B. Reduced vision

C. Painful red eye in a contact lens wearer

D. Watery discharge with a cold

E. Eye pain

F. Itching with sneezing

Swipe for answer →

Answer 26

A, B, C, E

- **True photophobia**
- **Reduced vision**
- **Painful red eye in a contact lens wearer**
- **Eye pain**

Why: Pain, vision loss, photophobia, corneal problems, or a contact lens wearer with pain are NOT simple pink eye.

Eye

Question 27 of 100

SINGLE ANSWER

Which medication may be used to CLOSE a PDA in a selected premature infant?

A. Indomethacin or ibuprofen

B. Digoxin

C. Dexamethasone

D. Prostaglandin E1

Swipe for answer →

Answer 27

A

- **Indomethacin or ibuprofen**

Why: "Indomethacin Interrupts (closes); Prostaglandin keeps it Patent."

PDA/obstructive

SELECT ALL THAT APPLY

Which are signs of heart failure in an infant?

A. Poor weight gain

B. Sweating during feeds

C. Hepatomegaly

D. Tachypnea

E. Tachycardia

F. Bradycardia

G. Ankle edema as the earliest sign

Swipe for answer →

Answer 28

A, B, C, D, E

- **Poor weight gain**
- **Sweating during feeds**
- **Hepatomegaly**
- **Tachypnea**
- **Tachycardia**

Why: "Feeding is the infant's treadmill." Peripheral edema is less prominent than in adults.

CHD basics

Question 29 of 100

SINGLE ANSWER

What is the first-line amoxicillin dose for AOM?

A. 25 mg/kg/day divided TID

B. 80–90 mg/kg/day divided BID

C. 50 mg/kg once daily

D. 20 mg/kg/dose BID

Swipe for answer →

Answer 29

B

- **80–90 mg/kg/day divided BID**

Why: High-dose amoxicillin for AOM differs from the GAS regimen (50 mg/kg once daily).

Otitis media

Question 30 of 100

SINGLE ANSWER

Which heart sound is the classic finding in an atrial septal defect?

A. Wide, fixed split S2

B. Diastolic murmur at the apex

C. Continuous machinery murmur

D. Harsh holosystolic murmur at the lower left sternal border

Swipe for answer →

Answer 30

A

- **Wide, fixed split S2**

Why: ASD → "Always Split, Doesn't change." Increased right-sided flow delays pulmonic valve closure.

ASD/VSD

Question 31 of 100

SELECT ALL THAT APPLY

Which findings suggest epiglottitis rather than croup?

A. Muffled voice

B. Drooling

C. Gradual onset after a cold

D. Abrupt high fever with toxic appearance

E. Barking seal-like cough

F. Tripod positioning

Swipe for answer →

Answer 31

A, B, D, F

- **Muffled voice**
- **Drooling**
- **Abrupt high fever with toxic appearance**
- **Tripod positioning**

Why: Epiglottitis = 4 D's (Drooling, Dysphagia, Dysphonia, Distress). Little barking cough. Don't inspect the throat.

Croup

Question 32 of 100

SINGLE ANSWER

Which treatment does the nurse expect for Kawasaki disease?

A. Benzathine penicillin monthly

B. Amoxicillin plus ibuprofen

C. Oral prednisone only

D. IVIG 2 g/kg plus aspirin

Swipe for answer →

Answer 32

D

- **IVIG 2 g/kg plus aspirin**

Why: IVIG within the first 10 days reduces coronary complications. Kawasaki is the exception to avoiding aspirin.

Kawasaki

SINGLE ANSWER

Which findings are classic for a significant patent ductus arteriosus?

A. Fixed split S2

B. Boot-shaped heart on x-ray

C. Continuous machinery murmur with bounding pulses

D. Weak femoral pulses

Swipe for answer →

Answer 33

C

- **Continuous machinery murmur with bounding pulses**

Why: PDA = "machinery." Wide pulse pressure and bounding pulses.

PDA/obstructive

Question 34 of 100

SINGLE ANSWER

Which child needs dental endocarditis prophylaxis for a qualifying procedure?

A. A child with an uncomplicated ASD

B. A child with mitral valve prolapse alone

C. A child with a small isolated VSD

D. A child with a prosthetic heart valve

Swipe for answer →

Answer 34

D

- **A child with a prosthetic heart valve**

Why: Prophylaxis is limited to the highest-risk patients (prosthetic valve, prior IE, selected cyanotic/repaired CHD).

Other cardiac

SINGLE ANSWER

A child received IVIG 2 g/kg for Kawasaki disease. How long should MMR and varicella be delayed?

A. 6 months

B. They are permanently contraindicated

C. 11 months

D. 1 month

Swipe for answer →

Answer 35

C

- **11 months**

Why: "2 grams → wait 11 months." Non-live vaccines do not need to wait.

Kawasaki

SINGLE ANSWER

An 8-year-old has symptoms 3 days/week, wakes at night 3 times/month, uses albuterol 3 days/week, has minor activity limitation, and an FEV1 of 85%. Classify the severity:

A. Severe persistent

B. Moderate persistent

C. Mild persistent

D. Intermittent

Swipe for answer →

Answer 36

C

- **Mild persistent**

Why: More than 2 days/week but not daily, and 3–4 nights/month = mild persistent.

Asthma

Question 37 of 100

SINGLE ANSWER

A newborn with d-transposition of the great arteries is severely cyanotic. Which medication does the nurse anticipate?

A. Aspirin

B. Furosemide

C. Prostaglandin E1 to keep the ductus open

D. Indomethacin to close the ductus

Swipe for answer →

Answer 37

C

- **Prostaglandin E1 to keep the ductus open**

Why: TGA = "Two separate circles" that need mixing. Never close that PDA.

Cyanotic CHD

SINGLE ANSWER

A teen faints while playing basketball. A systolic ejection murmur is heard. What is the most appropriate response?

A. Reassure, since syncope is common in teens

B. Recheck at next year's physical

C. Hold sports clearance and arrange a cardiac evaluation

D. Clear for sports with more hydration

Swipe for answer →

Answer 38

C

- **Hold sports clearance and arrange a cardiac evaluation**

Why: "Faints while running = Referral." Consider aortic stenosis or HCM.

PDA/obstructive

Question 39 of 100

SINGLE ANSWER

A child being treated for AOM develops fever, redness and swelling behind the ear, and an ear that sticks out. The nurse suspects:

A. Mastoiditis requiring urgent evaluation

B. Otitis media with effusion

C. Normal healing

D. Otitis externa

Swipe for answer →

Answer 39

A

- **Mastoiditis requiring urgent evaluation**

Why: Postauricular swelling and a protruding pinna = mastoiditis → urgent hospital/ENT care.

Otitis media

Question 40 of 100

SINGLE ANSWER

What causes functional closure of the foramen ovale after birth?

A. Increased left atrial pressure

B. Decreased systemic resistance

C. Prostaglandin E1 release

D. Increased pulmonary vascular resistance

Swipe for answer →

Answer 40

A

- **Increased left atrial pressure**

Why: Lungs expand → PVR falls, cord clamping raises systemic resistance → left atrial pressure rises and pushes the flap closed.

Fetal circulation

Question 41 of 100

SINGLE ANSWER

A child has had a firm, painless eyelid nodule for 3 weeks. The most likely diagnosis and initial care are:

A. Chalazion; squeeze to drain it

B. Orbital cellulitis; IV antibiotics

C. Hordeolum; oral antibiotics

D. Chalazion; warm compresses and lid hygiene

Swipe for answer →

Answer 41

D

- **Chalazion; warm compresses and lid hygiene**

Why: "A Stye Stings, a Chalazion is Chill." A chalazion is sterile, so antibiotics are not routine. Never squeeze.

Eye

Question 42 of 100

SINGLE ANSWER

A child gets a household cleaner splashed in the eye. What is the first action?

A. Take a full history first

B. Begin immediate, copious irrigation

C. Apply an eye patch

D. Check visual acuity before anything else

Swipe for answer →

Answer 42

B

- **Begin immediate, copious irrigation**

Why: Chemical injury: irrigate FIRST. Do not delay irrigation for a history.

Vision

Question 43 of 100

SELECT ALL THAT APPLY

A 7-week-old has pertussis. Which findings might the nurse see?

A. Apnea

B. Cyanosis

C. Vomiting after coughing

D. A whoop with every cough

E. Feeding difficulty

F. A barking seal-like cough

Swipe for answer →

Answer 43

A, B, C, E

- **Apnea**
- **Cyanosis**
- **Vomiting after coughing**
- **Feeding difficulty**

Why: Infants often have NO whoop; they may have apnea, cyanosis, and feeding trouble.

Pertussis

SELECT ALL THAT APPLY

Which are diagnostic features of Kawasaki disease (with fever of 5 or more days)?

A. Purulent eye discharge

B. Migratory polyarthritis

C. Strawberry tongue and cracked lips

D. Red, swollen hands and feet

E. Cervical lymph node of 1.5 cm or more

F. Polymorphous rash

G. Both eyes red without discharge

Swipe for answer →

Answer 44

C, D, E, F, G

- **Strawberry tongue and cracked lips**
- **Red, swollen hands and feet**
- **Cervical lymph node of 1.5 cm or more**
- **Polymorphous rash**
- **Both eyes red without discharge**

Why: CRASH and BURN: Conjunctivitis (nonpurulent), Rash, Adenopathy, Strawberry tongue, Hands/feet + fever (BURN). Migratory arthritis = rheumatic fever.

Kawasaki

Question 45 of 100

SINGLE ANSWER

Two weeks after a strep infection, a child has tea-colored urine, puffy eyes, and an elevated blood pressure. The nurse suspects:

A. Kawasaki disease

B. Acute rheumatic fever

C. Poststreptococcal glomerulonephritis

D. Peritonsillar abscess

Swipe for answer →

Answer 45

C

- **Poststreptococcal glomerulonephritis**

Why: PSGN = "Cola, Puffy, Pressure": dark urine, edema, and high BP after strep.

Strep complications

Question 46 of 100

SINGLE ANSWER

A child has palpitations every day. Which test is most appropriate?

A. Echocardiogram only

B. Holter monitor

C. Event monitor for several months

D. Cardiac catheterization

Swipe for answer →

Answer 46

B

- **Holter monitor**

Why: "Holter = Hold on for a day" (frequent symptoms). "Event = Every now and then" (rare symptoms).

Cardiac tests

Question 47 of 100

SINGLE ANSWER

Which test confirms the location and size of a suspected VSD?

A. ECG

B. Chest x-ray

C. Echocardiogram with Doppler

D. Pulse oximetry

Swipe for answer →

Answer 47

C

- **Echocardiogram with Doppler**

Why: "Echo = Eyes (anatomy); ECG = Electricity." An ECG cannot show a hole.

Cardiac tests

SINGLE ANSWER

During a well-child exam, the nurse notes a white pupillary reflex in one eye. What is the priority?

A. Reassure the family because the child has no pain

B. Start antibiotic eye drops

C. Recheck at the next well visit

D. Prompt ophthalmology referral to evaluate for retinoblastoma

Swipe for answer →

Answer 48

D

- **Prompt ophthalmology referral to evaluate for retinoblastoma**

Why: Leukocoria = retinoblastoma (or cataract) until proven otherwise. Absence of pain is not reassuring.

Vision

Question 49 of 100

SINGLE ANSWER

A child has a higher blood pressure in the arms than in the legs, with weak, delayed femoral pulses. The nurse suspects:

A. Coarctation of the aorta

B. Pulmonary stenosis

C. Patent ductus arteriosus

D. Atrial septal defect

Swipe for answer →

Answer 49

A

- **Coarctation of the aorta**

Why: Coarctation = "Strong upstairs, weak downstairs."

PDA/obstructive

Question 50 of 100

SINGLE ANSWER

Which instruction is correct for a child's nosebleed?

A. Pinch for 1 minute, then check

B. Lie flat with ice on the forehead

C. Tilt the head back and pinch the bridge

D. Sit up, lean forward, and pinch the soft part of the nose for 10 minutes

Swipe for answer →

Answer 50

D

- **Sit up, lean forward, and pinch the soft part of the nose for 10 minutes**

Why: Lean FORWARD and pinch the soft alae continuously for at least 10 minutes. Never tilt back.

School and teaching

Question 51 of 100

SINGLE ANSWER

An infant with TOF suddenly becomes deeply cyanotic and breathes rapidly while crying. The nurse should first:

A. Give albuterol

B. Place the infant in a knee-chest position

C. Lay the infant flat and supine

D. Start a feeding to calm the infant

Swipe for answer →

Answer 51

B

- **Place the infant in a knee-chest position**

Why: Tet spell → knee-chest (raises systemic resistance and pushes blood to the lungs), calm the infant, give O2, call for help.

Cyanotic CHD

SELECT ALL THAT APPLY

Which return-to-school statements are correct?

A. Ear infection alone does not require exclusion

B. Fifth disease: stay home until the rash fades

C. Pink eye: always 24 hours of antibiotic drops first

D. Strep: afebrile and 24 hours of antibiotics

E. Pertussis: after 5 full days of antibiotics

F. Chickenpox: when all lesions are crusted

Swipe for answer →

Answer 52

A, D, E, F

- **Ear infection alone does not require exclusion**
- **Strep: afebrile and 24 hours of antibiotics**
- **Pertussis: after 5 full days of antibiotics**
- **Chickenpox: when all lesions are crusted**

Why: Fifth disease is most contagious BEFORE the rash. There is no universal 24-hour-drops rule for conjunctivitis.

School and teaching

SELECT ALL THAT APPLY

A child with a sore throat is being assessed. Which findings mean the AIRWAY now takes priority?

A. Drooling

B. Tonsillar exudate

C. Inability to swallow secretions

D. Stridor

E. Temperature of 38.3°C

F. Toxic appearance

Swipe for answer →

Answer 53

A, C, D, F

- **Drooling**
- **Inability to swallow secretions**
- **Stridor**
- **Toxic appearance**

Why: Drooling, stridor, can't swallow secretions, toxic look, neck swelling, or distress change the urgency. Stop the throat exam and get urgent help.

Strep complications

Question 54 of 100

SINGLE ANSWER

After a cardiac catheterization through the femoral artery, which assessment is the priority?

A. Pupil reactivity

B. Bowel sounds

C. Access-site bleeding and pulses distal to the site

D. Urine specific gravity

Swipe for answer →

Answer 54

C

- **Access-site bleeding and pulses distal to the site**

Why: After a cath, monitor the site for bleeding and check distal pulses and perfusion.

Cardiac tests

Question 55 of 100

SINGLE ANSWER

Which vaccines are routinely given at 11–12 years?

A. MMR and varicella

B. Hib and PCV

C. DTaP, IPV, and MMR

D. Tdap, MenACWY, and HPV

Swipe for answer →

Answer 55

D

- **Tdap, MenACWY, and HPV**

Why: "Teens Meet HPV." HPV may start at age 9; MenACWY booster at 16.

Immunizations

SELECT ALL THAT APPLY

Which murmur features suggest a PATHOLOGIC murmur?

A. Harsh holosystolic quality

B. Soft, musical, louder lying down

C. Diastolic timing

D. A palpable thrill

E. Grade 3 or louder

F. Gets louder when standing

Swipe for answer →

Answer 56

A, C, D, E, F

- **Harsh holosystolic quality**
- **Diastolic timing**
- **A palpable thrill**
- **Grade 3 or louder**
- **Gets louder when standing**

Why: "Diastolic is always Dangerous." Louder standing/Valsalva suggests HCM. The soft musical supine murmur is Still's (innocent).

Murmurs

SINGLE ANSWER

A 4-year-old with nonsevere AOM is prescribed amoxicillin. What is the typical duration?

A. 14 days

B. 10 days

C. 5 days

D. 7 days

Swipe for answer →

Answer 57

D

- **7 days**

Why: "10–7–5": under 2 or severe = 10 days; age 2–5 = 7 days; age 6 and older = 5–7 days.

Otitis media

SINGLE ANSWER

A 6-year-old has fever and abdominal pain with a benign abdominal exam. What should the nurse do next?

A. Reassure the family that it is a stomach virus

B. Arrange an abdominal ultrasound

C. Assess respiratory rate and auscultate the lungs

D. Give ibuprofen and discharge

Swipe for answer →

Answer 58

C

- **Assess respiratory rate and auscultate the lungs**

Why: Lower-lobe pneumonia can present as belly pain. Always examine the lungs in a febrile child with abdominal complaints.

Pneumonia

SELECT ALL THAT APPLY

Which findings suggest a child's asthma is NOT well controlled?

A. Two oral steroid bursts this year

B. Peak flow at 90% of personal best

C. Waking at night with cough

D. Limiting activity because of symptoms

E. Using albuterol more than 2 days a week

F. Using the inhaled steroid every day

Swipe for answer →

Answer 59

A, C, D, E

- **Two oral steroid bursts this year**
- **Waking at night with cough**
- **Limiting activity because of symptoms**
- **Using albuterol more than 2 days a week**

Why: Frequent rescue use, night waking, activity limits, and steroid-requiring exacerbations all signal poor control.

Asthma

Question 60 of 100

SELECT ALL THAT APPLY

Which vaccines are school-entry boosters at age 4–6 years?

A. IPV

B. DTaP

C. HPV

D. MMR

E. Varicella

F. MenACWY

Swipe for answer →

Answer 60

A, B, D, E

- **IPV**
- **DTaP**
- **MMR**
- **Varicella**

Why: School entry = "DIMV." HPV and MenACWY are at 11–12 years.

Immunizations

Question 61 of 100

SINGLE ANSWER

An 18-kg child is ordered ibuprofen 10 mg/kg/dose. The suspension is 100 mg/5 mL. How many mL?

A. 3.6 mL

B. 18 mL

C. 9 mL

D. 4.5 mL

Swipe for answer →

Answer 61

C

- **9 mL**

Why: 18 × 10 = 180 mg. 180 × 5 ÷ 100 = 9 mL.

Dose calculation

SELECT ALL THAT APPLY

Which are MAJOR Jones criteria for acute rheumatic fever?

A. Elevated ESR

B. Fever

C. Sydenham chorea

D. Erythema marginatum

E. Carditis

F. Migratory polyarthritis

G. Subcutaneous nodules

Swipe for answer →

Answer 62

C, D, E, F, G

- **Sydenham chorea**
- **Erythema marginatum**
- **Carditis**
- **Migratory polyarthritis**
- **Subcutaneous nodules**

Why: JONES = Joints, ♥ carditis, Nodules, Erythema marginatum, Sydenham chorea. Fever and ESR are minor (FEAR).

Rheumatic fever

SINGLE ANSWER

A 7-year-old has sudden sore throat, fever of 39.1°C, tender anterior cervical nodes, and palatal petechiae. No cough or runny nose. The rapid strep test is negative. What should the nurse anticipate next?

A. Send a backup throat culture

B. Reassure the family that the illness is viral

C. Start amoxicillin without further testing

D. Draw an ASO titer to confirm strep

Swipe for answer →

Answer 63

A

- **Send a backup throat culture**

Why: A negative rapid antigen test in a symptomatic child aged 3 or older needs a backup throat culture. ASO shows past strep, not acute infection.

Strep pharyngitis

SELECT ALL THAT APPLY

Which are appropriate for otitis media with effusion (OME) in a child without special risks?

A. Antibiotics

B. Antihistamines

C. Decongestants

D. Earlier hearing evaluation if the child has developmental risk

E. Watchful waiting for about 3 months

F. A hearing evaluation if fluid persists

Swipe for answer →

Answer 64

D, E, F

- **Earlier hearing evaluation if the child has developmental risk**
- **Watchful waiting for about 3 months**
- **A hearing evaluation if fluid persists**

Why: OME = "Fluid without Fire." No routine antibiotics, antihistamines, decongestants, or steroids.

Ear and hearing

Question 65 of 100

SINGLE ANSWER

Which otoscopic finding best supports a diagnosis of acute otitis media?

A. Clear fluid behind the TM without symptoms

B. A red TM in a crying child

C. Ear tugging

D. Moderate to severe bulging of the tympanic membrane

Swipe for answer →

Answer 65

D

- **Moderate to severe bulging of the tympanic membrane**

Why: "Bulge = AOM. Red alone = Rubbish." Crying reddens the TM, and tugging is nonspecific.

Otitis media

SELECT ALL THAT APPLY

Which medications are used for a moderate to severe acute asthma exacerbation?

A. Salmeterol

B. Albuterol

C. Oxygen if hypoxemic

D. Oral or IV corticosteroid

E. Ipratropium

F. Montelukast

Swipe for answer →

Answer 66

B, C, D, E

- **Albuterol**
- **Oxygen if hypoxemic**
- **Oral or IV corticosteroid**
- **Ipratropium**

Why: SABA plus ipratropium, a systemic steroid, and O2 when indicated. Montelukast and salmeterol are not rescue drugs.

Asthma

Question 67 of 100

SINGLE ANSWER

A 25-kg child with GAS is ordered cephalexin 20 mg/kg/dose BID. What is the correct amount?

A. 1,000 mg per dose

B. 250 mg per dose, twice daily

C. 500 mg per dose, twice daily

D. 500 mg per day total

Swipe for answer →

Answer 67

C

- **500 mg per dose, twice daily**

Why: "Per DOSE → Don't divide." 25 × 20 = 500 mg each dose (max 500 mg/dose).

Dose calculation

SINGLE ANSWER

A teen's murmur gets LOUDER when standing and during Valsalva. This suggests:

A. Hypertrophic cardiomyopathy

B. Normal physiologic split

C. Still's murmur

D. Venous hum

Swipe for answer →

Answer 68

A

- **Hypertrophic cardiomyopathy**

Why: HCM gets louder with standing/Valsalva (less filling). Most innocent murmurs get softer.

Murmurs

Question 69 of 100

SINGLE ANSWER

A child with pertussis started azithromycin. When may the child return to school?

A. When the cough completely stops

B. After 24 hours of antibiotics

C. After 10 days of antibiotics

D. After 5 full days of effective antibiotics

Swipe for answer →

Answer 69

D

- **After 5 full days of effective antibiotics**

Why: "5 days on drugs, 21 days without" (untreated: 21 days after cough onset).

Pertussis

Question 70 of 100

SINGLE ANSWER

A murmur is loud and accompanied by a palpable thrill. What is the Levine grade?

A. Grade VI

B. Grade III

C. Grade II

D. Grade IV

Swipe for answer →

Answer 70

D

- **Grade IV**

Why: "4 = you can FEEL it." Grade III is loud without a thrill; VI is heard with the stethoscope off the chest.

Murmurs

SELECT ALL THAT APPLY

A school-age child has a sore throat. Which findings make a VIRAL cause more likely than group A strep?

A. Oral ulcers

B. Palatal petechiae

C. Rhinorrhea

D. Conjunctivitis

E. Tender anterior cervical nodes

F. Hoarseness

G. Cough

Swipe for answer →

Answer 71

A, C, D, F, G

- **Oral ulcers**
- **Rhinorrhea**
- **Conjunctivitis**
- **Hoarseness**
- **Cough**

Why: "Looks like a COLD → not strep": Cough, Oral ulcers, Laryngitis/hoarseness, Drippy nose, plus pink eye. Petechiae and tender anterior nodes fit GAS.

Strep pharyngitis

SINGLE ANSWER

A 16-year-old has fever, exudative pharyngitis, marked fatigue, posterior cervical nodes, and splenomegaly. What is the priority teaching?

A. Take amoxicillin for 10 days

B. Avoid contact sports until the spleen is evaluated

C. Expect a tonsillectomy

D. Stay home from school for 21 days

Swipe for answer →

Answer 72

B

- **Avoid contact sports until the spleen is evaluated**

Why: "Front nodes = Strep, Back nodes = Mono." An enlarged spleen can rupture, so avoid contact sports.

Strep complications

SINGLE ANSWER

A 3-day-old has profuse purulent eye discharge with marked swelling (chemosis). The nurse suspects:

A. Chlamydial conjunctivitis treated with topical ointment

B. Gonococcal conjunctivitis needing urgent systemic ceftriaxone

C. Allergic conjunctivitis

D. Blocked tear duct

Swipe for answer →

Answer 73

B

- **Gonococcal conjunctivitis needing urgent systemic ceftriaxone**

Why: "Gonorrhea Goes fast" (days 2–5); it threatens the cornea. "Chlamydia Comes later" (days 5–12, oral treatment).

Vision

SELECT ALL THAT APPLY

Which are components of Tetralogy of Fallot?

A. Right ventricular hypertrophy

B. Coarctation of the aorta

C. Ventricular septal defect

D. Atrial septal defect

E. Pulmonary stenosis

F. Overriding aorta

Swipe for answer →

Answer 74

A, C, E, F

- **Right ventricular hypertrophy**
- **Ventricular septal defect**
- **Pulmonary stenosis**
- **Overriding aorta**

Why: TOF = PROV: Pulmonary stenosis, RVH, Overriding aorta, VSD.

Cyanotic CHD

SINGLE ANSWER

A 30-kg child with confirmed GAS cannot reliably take oral medication. What is the correct single IM dose of benzathine penicillin G?

A. 600,000 units

B. 1.2 million units

C. 2.4 million units

D. 300,000 units

Swipe for answer →

Answer 75

B

- **1.2 million units**

Why: 27 kg is the dividing line: under 27 kg = 600,000 units; 27 kg or more = 1.2 million units.

Strep pharyngitis

SELECT ALL THAT APPLY

Which situations are generally NOT reasons to delay routine vaccines?

A. Moderate or severe acute illness

B. An immunocompromised family member at home

C. Mild cold without fever

D. Anaphylaxis to a previous dose

E. Currently taking antibiotics

F. Using an inhaled steroid

Swipe for answer →

Answer 76

B, C, E, F

- **An immunocompromised family member at home**
- **Mild cold without fever**
- **Currently taking antibiotics**
- **Using an inhaled steroid**

Why: Mild illness, antibiotics, inhaled steroids, and household immunocompromise are not reasons to wait. Anaphylaxis = contraindication; moderate/severe illness = precaution.

Immunizations

SINGLE ANSWER

A child received MMR. Two weeks later, varicella vaccine was given. How should the nurse interpret the varicella dose?

A. It is not valid; injectable live vaccines must be given the same day or at least 28 days apart

B. It is valid; live vaccines can be given at any interval

C. Only the MMR needs to be repeated

D. It is valid because MMR and varicella are different viruses

Swipe for answer →

Answer 77

A

- **It is not valid; injectable live vaccines must be given the same day or at least 28 days apart**

Why: "Live shots: same day or wait a month (28 days)." The second live vaccine given too early should be repeated.

Immunizations

SINGLE ANSWER

A 9-year-old has a cough after a cold, scattered rhonchi, no focal findings, and yellow sputum. The appropriate management is:

A. Amoxicillin for 10 days

B. Supportive care; antibiotics are not routinely indicated

C. Chest x-ray and CBC before deciding

D. Azithromycin for 5 days

Swipe for answer →

Answer 78

B

- **Supportive care; antibiotics are not routinely indicated**

Why: Acute bronchitis is usually viral. Purulent sputum alone does not justify antibiotics.

Other respiratory

SINGLE ANSWER

A child who developed severe pneumonia shortly after influenza is most likely infected with which organism, in addition to pneumococcus?

A. Staphylococcus aureus, including MRSA

B. Chlamydia trachomatis

C. Mycoplasma pneumoniae

D. Group B Streptococcus

Swipe for answer →

Answer 79

A

- **Staphylococcus aureus, including MRSA**

Why: After the flu → think Staph (including MRSA) and pneumococcus.

Pneumonia

Question 80 of 100

SINGLE ANSWER

A 2-year-old started coughing suddenly while eating peanuts. Now there is wheezing and decreased breath sounds on the right only. The chest x-ray is normal. The next step is:

A. Reassurance because the x-ray is normal

B. Prompt specialist referral for bronchoscopy

C. Amoxicillin for pneumonia

D. An albuterol trial for new asthma

Swipe for answer →

Answer 80

B

- **Prompt specialist referral for bronchoscopy**

Why: "Sudden + one Side" = foreign body. Food is radiolucent, so a normal x-ray does not rule it out.

Foreign body

SINGLE ANSWER

A child is nearsighted (myopic). Which statement is correct?

A. Distant objects are blurry because light focuses in front of the retina

B. Near objects are blurry because light focuses behind the retina

C. Both eyes have unequal refractive errors

D. The eyes are misaligned

Swipe for answer →

Answer 81

A

- **Distant objects are blurry because light focuses in front of the retina**

Why: "MY-opia: I only see what's near MY face." Hyperopia = farsighted; anisometropia = unequal; strabismus = misaligned.

Vision

Question 82 of 100

SINGLE ANSWER

In fetal circulation, the foramen ovale shunts blood from:

A. Umbilical vein past the liver

B. Right atrium to left atrium

C. Left ventricle to right ventricle

D. Pulmonary artery to aorta

Swipe for answer →

Answer 82

B

- **Right atrium to left atrium**

Why: Foramen Ovale = "Open door between the atria" (RA → LA). Ductus arteriosus = PA → aorta. Ductus venosus bypasses the liver.

Fetal circulation

Question 83 of 100

SINGLE ANSWER

A 20-kg child took prednisone 2 mg/kg/day for 3 weeks and stopped yesterday. MMR is due. The nurse should:

A. Give MMR today

B. Wait at least 1 month after stopping before giving MMR

C. Give MMR at half dose

D. Never give MMR to this child

Swipe for answer →

Answer 83

B

- **Wait at least 1 month after stopping before giving MMR**

Why: Steroid "2–20–14": 2 mg/kg/day or 20 mg/day or more for 14 days or more → delay live vaccines at least 1 month.

Immunizations

SELECT ALL THAT APPLY

Which findings suggest ORBITAL rather than preseptal cellulitis?

A. Pain with eye movement

B. Restricted eye movement

C. Decreased vision

D. Normal visual acuity

E. Proptosis

F. Swollen red eyelid with normal eye movement

Swipe for answer →

Answer 84

A, B, C, E

- **Pain with eye movement**
- **Restricted eye movement**
- **Decreased vision**
- **Proptosis**

Why: Orbital = 4 P's: Pain, Proptosis, Poor vision, Paralysis → emergency IV antibiotics.

Eye

SELECT ALL THAT APPLY

Which interventions are NOT routinely recommended for typical bronchiolitis?

A. Oxygen for hypoxemia

B. Antibiotics

C. Systemic steroids

D. Gentle nasal suction

E. Chest physiotherapy

F. Albuterol

Swipe for answer →

Answer 85

B, C, E, F

- **Antibiotics**
- **Systemic steroids**
- **Chest physiotherapy**
- **Albuterol**

Why: Bronchiolitis = 3 S's: Suction, Sips, Saturation. Supportive care only.

Other respiratory

SINGLE ANSWER

A 4-year-old reads the 20/50 line with each eye. What should the nurse do?

A. Pass the child, because 20/50 is normal at 4

B. Refer only if the child has symptoms

C. Retest in 2 years

D. Refer, because the passing threshold at age 4 is 20/40

Swipe for answer →

Answer 86

D

- **Refer, because the passing threshold at age 4 is 20/40**

Why: AAPOS: age 3 = 20/50, age 4 = 20/40, age 5+ = 20/32. "Child grows, number shrinks."

Vision

Question 87 of 100

SINGLE ANSWER

A toddler with presumed croup has high fever, looks toxic, and does not improve with dexamethasone and epinephrine. The nurse suspects:

A. Mild viral croup

B. Bacterial tracheitis

C. Allergic rhinitis

D. Bronchiolitis

Swipe for answer →

Answer 87

B

- **Bacterial tracheitis**

Why: High fever, toxicity, and poor response to standard croup treatment suggest bacterial tracheitis.

Croup

SINGLE ANSWER

A 4-year-old has abrupt high fever, drooling, a muffled voice, and is sitting in a tripod position without a barking cough. The nurse's priority is to:

A. Keep the child calm with the parent and prepare for emergency airway care

B. Send the child to radiology alone for a neck x-ray

C. Inspect the throat with a tongue blade

D. Obtain a throat culture

Swipe for answer →

Answer 88

A

- **Keep the child calm with the parent and prepare for emergency airway care**

Why: Epiglottitis = 4 D's (Drooling, Dysphagia, Dysphonia, Distress). Do not inspect the throat. Airway comes first.

Croup

SINGLE ANSWER

An infant received MMR at 9 months before international travel. What is the plan?

A. No further MMR doses are needed

B. The travel dose counts as dose 1

C. Give one more dose at 4–6 years only

D. Still give 2 routine MMR doses starting at 12 months

Swipe for answer →

Answer 89

D

- **Still give 2 routine MMR doses starting at 12 months**

Why: A dose before 12 months does not count toward the routine 2-dose series.

Immunizations

SINGLE ANSWER

The most serious complication of Kawasaki disease is:

A. Kidney failure

B. Hearing loss

C. Mitral valve damage

D. Coronary artery aneurysm

Swipe for answer →

Answer 90

D

- **Coronary artery aneurysm**

Why: "Kawasaki Crowns the Coronaries." (Rheumatic fever targets valves.)

Kawasaki

Question 91 of 100

SELECT ALL THAT APPLY

Which vaccines are LIVE vaccines?

A. Rotavirus

B. IPV

C. Intranasal influenza (LAIV)

D. MMR

E. DTaP

F. Varicella

Swipe for answer →

Answer 91

A, C, D, F

- **Rotavirus**
- **Intranasal influenza (LAIV)**
- **MMR**
- **Varicella**

Why: Injected/nasal live vaccines: same day or 28+ days apart. Oral rotavirus is the exception to that spacing rule.

Immunizations

Question 92 of 100

SINGLE ANSWER

A child's personal best peak flow is 300 L/min. Today's reading is 180 L/min. Which zone is this?

A. Cannot be determined

B. Red zone

C. Green zone

D. Yellow zone

Swipe for answer →

Answer 92

D

- **Yellow zone**

Why: 180/300 = 60%. Green is 80% or more, yellow 50–79%, red under 50%.

Asthma

Question 93 of 100

SINGLE ANSWER

A child weighs 22 kg. Acetaminophen is ordered at 15 mg/kg/dose. The suspension is 160 mg/5 mL. How many mL per dose?

A. 10.3 mL

B. 6.9 mL

C. 15 mL

D. 3.3 mL

Swipe for answer →

Answer 93

A

- **10.3 mL**

Why: 22 × 15 = 330 mg. 330 × 5 ÷ 160 = 10.3 mL.

Dose calculation

SINGLE ANSWER

A 12-year-old got HPV dose 1, and dose 2 was given 4 months later. What is needed?

A. Restart the series

B. Two more doses

C. Another dose, because doses less than 5 months apart are not valid for the 2-dose series

D. Nothing; the series is complete

Swipe for answer →

Answer 94

C

- **Another dose, because doses less than 5 months apart are not valid for the 2-dose series**

Why: Started before 15 = 2 doses 6–12 months apart; if less than 5 months apart, a third dose is needed.

Immunizations

Question 95 of 100

SELECT ALL THAT APPLY

Which statements about pediatric fever medicines are correct?

A. Ibuprofen is generally for infants 6 months and older

B. Acetaminophen may be given every 2 hours

C. Ibuprofen 5–10 mg/kg every 6–8 hours

D. Acetaminophen maximum 5 doses in 24 hours

E. Acetaminophen 10–15 mg/kg every 4–6 hours

F. Aspirin is a good routine fever reducer

Swipe for answer →

Answer 95

A, C, D, E

- **Ibuprofen is generally for infants 6 months and older**
- **Ibuprofen 5–10 mg/kg every 6–8 hours**
- **Acetaminophen maximum 5 doses in 24 hours**
- **Acetaminophen 10–15 mg/kg every 4–6 hours**

Why: Ibuprofen = "the 6s" (6 months, every 6–8 hrs). Aspirin carries a Reye syndrome risk except in Kawasaki.

Medications

SELECT ALL THAT APPLY

Which instructions are correct for a child using an inhaled corticosteroid (ICS)?

A. Use it during an acute attack for quick relief

B. Use it every day, even when feeling well

C. Growth will be monitored

D. Use a spacer with the metered-dose inhaler

E. Rinse the mouth and spit after use

F. Use a spacer with a dry-powder inhaler

Swipe for answer →

Answer 96

B, C, D, E

- **Use it every day, even when feeling well**
- **Growth will be monitored**
- **Use a spacer with the metered-dose inhaler**
- **Rinse the mouth and spit after use**

Why: ICS = Insurance (daily controller). Rinse and spit. Spacers go with MDIs, NOT dry-powder inhalers.

Asthma

SELECT ALL THAT APPLY

Which congenital heart defects are ACYANOTIC?

A. Coarctation of the aorta

B. Ventricular septal defect

C. Atrial septal defect

D. Patent ductus arteriosus

E. Tetralogy of Fallot

F. Transposition of the great arteries

Swipe for answer →

Answer 97

A, B, C, D

- **Coarctation of the aorta**
- **Ventricular septal defect**
- **Atrial septal defect**
- **Patent ductus arteriosus**

Why: Acyanotic = the "D's" (ASD, VSD, PDA) plus obstructive lesions (coarctation, AS, PS). Cyanotic = the 5 T's.

CHD basics

Question 98 of 100

SINGLE ANSWER

Which teaching about montelukast is most important?

A. Use it for acute attacks

B. Rinse the mouth after each dose

C. Report mood or behavior changes, including suicidal thoughts

D. It replaces the albuterol inhaler

Swipe for answer →

Answer 98

C

- **Report mood or behavior changes, including suicidal thoughts**

Why: "Montelukast = Mood warning" (boxed warning). It is not a rescue medication.

Asthma

SINGLE ANSWER

A 9-year-old has daytime symptoms 2 days/week, but wakes at night twice a week. The FEV1 is 82%. Classify the severity:

A. Moderate persistent

B. Intermittent

C. Severe persistent

D. Mild persistent

Swipe for answer →

Answer 99

A

- **Moderate persistent**

Why: "Worst feature wins." Night waking more than once a week (but not nightly) = moderate persistent.

Asthma

SINGLE ANSWER

A tall teen with long fingers, pectus excavatum, and a dislocated lens likely has Marfan syndrome. Which cardiac concern is the priority?

A. Aortic root dilation and dissection

B. Coarctation

C. Coronary aneurysm

D. Tetralogy of Fallot

Swipe for answer →

Answer 100

A

- **Aortic root dilation and dissection**

Why: Marfan → aortic root aneurysm/dissection, plus possible MVP.

Other cardiac